Dose-dense AC → fortnightly paclitaxel + weekly trastuzumab

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

AC · 14-day schedule

Active medicines: Doxorubicin + Cyclophosphamide

Emetogenic risk: High · AC

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1
Ondansetron	16 mg orally	Day 1 Individual dose / QT / hepatic review
Dexamethasone	12 mg orally	Day 1 No routine Days 2–4 dexamethasone in this NCCP AC antiemetic row
Olanzapine	5 mg orally	Days 1–4 Review sedation, falls, interactions and glucose
Aprepitant	80 mg orally daily	Days 2–3
Metoclopramide	10 mg orally up to three times daily	Subsequent days: duration and use to be confirmed Caution with olanzapine: additive extrapyramidal effects; adjust / avoid for individual contraindications

Additional phase-specific support

- This AC antiemetic script is replaced at the taxane transition.
- Growth-factor support is required for dose-dense AC. Specify the product, dose and days under local policy.
- Select the taxane support plan at the phase transition; do not continue the AC antiemetic course by default.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00316 v6 · p6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-25 · SACTCheck v0.84.18 · Regimen-level drafts; prescriber and oncology-pharmacy verification required